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REPORT

PT Name : **HARSHAD BHUSARE**
Ref By : **DR. SELF**
Reg No : **HL5401830813 / PUN104**
Barcode : **Hen183590**
INV : HIV 1- Quantitative PCR

Age : 30 Year | **Sex :** Male
Registered on : 30-08-2026 08:22 AM
Received on : 01-09-2026 06:08 PM
Reported on : 02-09-2026 01:57 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

HIV 1- QUANTITATIVE PCR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Human immunodeficiency virus 1 RNA Viral Load (Quantitative) PCR Method: (Real Time PCR)	< 39.1	IU/mL	. - .
RESULT	NOT DETECTED	.	. - .

Interpretation :

Result (IU/mL)	Comments
<39.1	HIV-1 RNA quantification cannot be used for diagnosing patients HIV-1 status. HIV-1 RNA test is meant to measure the viral load in known HIV-1 positive patients for clinical managements of the disease. But below the lower limit of linear range of the assay. The results below <39.1 IU/ml does not indicate that the patient is negative for HIV.
>39.1-10000000	HIV-1 RNA Detected within the linear range of the assay.
>10000000	HIV-1 RNA Detected above the linear range of the assay.

Clinical Utility:

- This assay is a quantitative assay used for monitoring patients on therapy and not qualitative assay used for screening.
- Measurement of HIV-1 RNA plasma levels (viral load) provides quantitative measurement of viremia and is used in conjunction with CD4+ T-cell counts. The test is used to assess prognosis, monitor progression of HIV-1 infection and effect of antiretroviral drug therapy.

Note:

- The same assay method should be used throughout the patient's care. It is not advisable to compare the viral loads between two different techniques.
- Wide variation in viral load have been observed and a clinically significant difference in viral load is 3-fold or 0.5 log10 IU/mL, changes due to use of different technologies/platforms for follow up testing. Hence, it is recommended to monitor patients using same technology.

Limitation of Assay:

PCR is a highly sensitive technique; common reasons for paradoxical results are contamination during specimen collection, selection of inappropriate specimen and inherent PCR inhibitors in the sample.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **RAHUL JADHAV**
Ref By : **DR. BETTERFEEL LAB**
Reg No : **HL1601831633 / MH004/MS121**
Barcode : **HEJ089030**
INV : Lactate Plasma

Age : 7 Year | **Sex :** Male
Registered on : 31-08-2026 12:17 AM
Received on : 31-08-2026 08:31 AM
Reported on : 01-09-2026 10:42 AM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

LACTATE PLASMA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LACTATE Method: Enzymatic	12.5	mg/dL	4.5 - 19.8

Clinical significance :

D-lactate is produced by bacteria residing in the colon when carbohydrates are not completely absorbed in the small intestine. When large amounts of D-lactate are present, individuals can experience metabolic acidosis, altered mental status (from drowsiness to coma), and a variety of other neurologic symptoms, particularly dysarthria and ataxia.

D-lactic acidosis is typically observed in patients with a malabsorptive disorder, such as short-bowel syndrome, or following a jejunoileal bypass. In addition, healthy children presenting with gastroenteritis may also develop the critical presentation of D-lactic acidosis.

Routine lactic acid determinations in blood will not reveal abnormalities because most lactic acid assays measure only L-lactate. Accordingly, D-lactate analysis must be specifically requested (eg, this test). However, as D-lactate is readily excreted in urine, DLAU / D-Lactate, Urine is the preferred specimen for D-lactate determinations.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **TRIPANA ACHARYA**
Ref By : **DR. EFANN-ECRC**
Reg No : **HL7801828400 / MH417**
Barcode : **HEI059395**
INV : Marijuana (CANNABINOIDS)

Age : 26 Year | **Sex :** Female
Registered on : 27-08-2026 02:51 PM
Received on : 27-08-2026 06:59 PM
Reported on : 30-08-2026 01:07 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MARIJUANA (CANNABINOIDS)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Marijuana Method: .	NEGATIVE	.	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR.SUMEET TAWRE**
Ref By : **DR. SELF**
Reg No : **HL3601832284 / RAJ41**
Barcode : **HEL002879**
INV : Microalbumin (Urine Random)

Age : 38 Year | **Sex :** Male
Registered on : 31-08-2026 03:39 PM
Received on : 31-08-2026 06:10 PM
Reported on : 31-08-2026 09:06 PM

SAMPLE COLLECTED AT :
STAR LAB

SAMPLE : Urine

MICROALBUMIN (URINE RANDOM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: PHOTOMETRIC	19.65	mg/dl	0.0 - 30.0

Clinical Information .

Microalbumin is a small amount of a protein called albumin. It is normally found in the blood. Creatinine is a normal waste product found in urine. A microalbumin creatinine ratio compares the amount of albumin to the amount of creatinine in your urine.

If there is any albumin in your urine, the amount can vary greatly throughout the day. But creatinine is released as a steady rate. Because of this, your health care provider can more accurately measure the amount of albumin by comparing it to the amount of creatinine in your urine. If albumin is found in your urine, it may mean you have a problem with your kidneys.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Ref By : **DR. SELF**
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INV : Microalbumin (Urine Random)

Age : 38 Year | **Sex :** Male
Registered on : 31-08-2026 03:39 PM
Received on : 31-08-2026 06:10 PM
Reported on : 31-08-2026 09:06 PM

SAMPLE COLLECTED AT :
STAR LAB

SAMPLE : Urine

MICROALBUMIN (URINE RANDOM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: PHOTOMETRIC	19.65	mg/dl	0.0 - 30.0

Clinical Information .

Microalbumin is a small amount of a protein called albumin. It is normally found in the blood. Creatinine is a normal waste product found in urine. A microalbumin creatinine ratio compares the amount of albumin to the amount of creatinine in your urine.

If there is any albumin in your urine, the amount can vary greatly throughout the day. But creatinine is released as a steady rate. Because of this, your health care provider can more accurately measure the amount of albumin by comparing it to the amount of creatinine in your urine. If albumin is found in your urine, it may mean you have a problem with your kidneys.

~~End of report~~

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REPORT

PT Name : **RAM BHARAN**
Ref By : **DR. SELF**
Reg No : **HL6001820930 / PB070**
Barcode : **MU077687**
INV : Mycobacterium TB PCR - Qualitative (FLUID)

Age : 84 Year | **Sex :** Male
Registered on : 20-08-2026 07:51 PM
Received on : 21-08-2026 08:47 AM
Reported on : 23-08-2026 04:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Body Fluid

MYCOBACTERIUM TB PCR - QUALITATIVE (FLUID)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Mycobacterium Tuberculosis Complex PCR Method: Realtime PCR	NOT DETECTED	.	. - .

Interpretation

Results	Comments
Mycobacterium tuberculosis complex & Non tuberculosis mycobacteria – Non Detected	Mycobacteria not detected in the sample provided.
Mycobacterium tuberculosis complex Detected	Infection likely with any of the following species – Mycobacterium tuberculosis, Mycobacterium africanum, Mycobacterium bovis, Mycobacterium bovis BCG, Mycobacterium microti, Mycobacterium canettii, Mycobacterium pinnipedii and Mycobacterium mungi.
Non tuberculosis mycobacteria Detected	Infection likely with mycobacterium other than tuberculosis complex.

Methodology: MTB/NTM detection is based on amplification of region of IS6110 and MPB64 gene of Mycoplasma tuberculosis (TB) and ITS gene of Nontuberculosis mycobacteria (NTM) and for a region of partial sequence of the Paulownia tomentosa external transcribed spacer(Internal control).

Clinical Significance: Tuberculosis, or TB, is an infectious bacterial disease caused by Mycobacterium tuberculosis, which most commonly affects the lungs. It is transmitted from person to person via droplets from the throat and lungs of people with the active respiratory disease. The classic symptoms of active TB infection are a chronic cough with blood tinged sputum, fever, night sweats, and weight loss. Infection of other organs causes a wide range of symptoms.

Recommendation: Positive PCR results should always be correlated with clinical status and history of the patient. A positive PCR finding in a symptomatic patient, irrespective of the findings on smear and/or culture, warrants active treatment for the Mycobacterium tuberculosis infection. This treatment should be instituted for the recommended length of time and should not be interrupted or discontinued before the entire period has been covered, in spite of symptomatic recovery being achieved partly early in the course of therapy.

~~End of report~~

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REPORT

PT Name : **KALPANA PAWAR**
Ref By : **DR. SELF**
Reg No : **HL4401830823 / MH026**
Barcode : **396709517**
INV : Protein 24 Hrs - Urinary

Age : 62 Year | **Sex :** Female
Registered on : 30-08-2026 08:28 AM
Received on : 30-08-2026 09:04 AM
Reported on : 30-08-2026 12:19 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

PROTEIN 24 HRS - URINARY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Total volume Method: .	900	ML/day	250 - 2400
Urinary Protein	**650.0**	mg/dL	0.0 - 15.0
PROTEIN 24 HRS Method: Spectrophotometry	**5850**	mg/dL 24 Hrs	0.0 - 150.0

Clinical significance :
A characteristic monoclonal band (M-spike) is often found in the urine of patients with monoclonal gammopathies. The initial identification of an M-spike or an area of restricted migration is followed by immunofixation to identify the immunoglobulin heavy chain and/or light chain.

Immunoglobulin free light chains as well as heavy chain fragments may be seen in the urine of patients with monoclonal gammopathies. The presence of a monoclonal light chain M-spike of greater than 1 g/24 hours is consistent with a diagnosis of multiple myeloma or macroglobulinemia.

The presence of a small amount of monoclonal light chain and proteinuria (total protein >3 g/24 hours) that is predominantly albumin is consistent with primary systemic amyloidosis (AL) or light chain deposition disease (LCDD).

Because patients with AL or LCDD may have elevated urinary protein without an identifiable M-spike, urine protein electrophoresis is not considered an adequate screen for these disorders and immunofixation is also recommended.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NILIMA SAWAKARE**	**Age :** 35 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. VRUSHALI TAJANE**	**Registered on :** 30-08-2026 02:17 PM	
Reg No	: **HL7101831243 / MH447**	**Received on** : 03-09-2026 10:47 AM	
Barcode	: **HEI055944**	**Reported on** : 04-09-2026 04:19 PM	
INV	: Roma Score (HE4 & CA125)		**SAMPLE :** Serum

ROMA SCORE (HE4 & CA125)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Cancer Ag 125 (CA 125),	7.28	U/mL	0.0 - 35
Human Epididymis Protein 4 (HE4),	28.0	pmol/L	20 - 1500
ROMA Value	1.90	%	- - - Premenopausal > or = 7.4 : High risk < 7.4 : Low risk Postmenopausal > or = 25.3 : High risk < 25.3 : Low risk

CA 125 and HE4 together improve ovarian cancer monitoring

- Either HE4 or CA 125 levels can be elevated in patients during therapy monitoring and recurrence monitoring.
- At disease progression, some patients show an elevated HE4 level earlier than an increase in CA 125 level, and others show an elevated CA 125 level earlier than an increase in HE4 level. Therefore, HE4 and CA 125 together can effectively monitor therapy response.
- As patients with elevated CA 125 level at diagnosis can switch to elevated HE4 level at follow up, and vice versa, testing for both HE4 and CA 125 can identify patients with recurrent disease that CA 125 alone would miss.

Clinical Information:

- Human epididymal protein 4 (HE4) is a biomarker that is highly elevated in epithelial ovarian cancer and has been shown to be involved in ovarian cancer cell proliferation and invasion 1-3
- HE4 increases the sensitivity of CA 125 in the detection of ovarian cancer in patients presenting with pelvic mass.
- Among women presenting with pelvic mass, the combined use of HE4 and CA 125 using ROMA (Risk of Ovarian Malignancy Algorithm) can increase the sensitivity and specificity of detecting ovarian cancer - even stages I/II ovarian cancer -than CA 125 alone.

NOTE :
ROMA accurately identifies 94% of the patients with pelvic mass as having ovarian cancer. ROMA has a higher sensitivity than CA 125 alone; among ten women with ovarian cancer, ROMA identifies one more patient than CA 125 alone (sensitivity: ROMA 90% vs. CA 125 79%)

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
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REPORT

PT Name : **MASTER MD HASAN**
Ref By : **DR. SELF**
Reg No : **HL7901832236 / MH037**
Barcode : **HEN170729**
INV : Salmonella Typhi (TYPHI DOT IgM)

Age : 15 Month | **Sex :** Male
Registered on : 31-08-2026 03:07 PM
Received on : 31-08-2026 03:34 PM
Reported on : 31-08-2026 07:01 PM

SAMPLE COLLECTED AT :
FOR BOOKING:-7021461972

SAMPLE : Serum

SALMONELLA TYPHI (TYPHI DOT IGM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Typhi DOT - IgM Method: ELISA	0.62	OD RATIO	0.00 - 0.90

CLINICAL SIGNIFICANCE

The Typhidot is easy to perform, and requires no special equipment or training of staff for interpretation of results. It will be a useful complementary test to blood culture and the Widal test in the diagnosis of typhoid fever.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **DIPTI DILIP RANE**
Ref By : **DR. HARDIK PATEL**
Reg No : **HL5201828796 / MH004/7/MH00K**
Barcode : **HED063013**
INV : Sodium Biocorbonet

Age : 58 Year | **Sex :** Female
Registered on : 27-08-2026 11:52 PM
Received on : 28-08-2026 10:55 AM
Reported on : 28-08-2026 04:43 PM

SAMPLE COLLECTED AT :
INDIPATHLAB

SAMPLE : Serum

SODIUM BIOCORBONET

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Bicarbonate Method: .	**19.36**	mmol/L	22.0 - 29.0

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR CHANDRAKANT GHOLAP**
Ref By : **DR. SELF**
Reg No : **HL6401825653 / MH392**
Barcode : **HEN151140**
INV : Sodium Random Urine

Age : 73 Year | **Sex :** Male
Registered on : 25-08-2026 11:39 AM
Received on : 25-08-2026 03:53 PM
Reported on : 25-08-2026 11:58 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

SODIUM RANDOM URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SODIUM Method: Electrolytes	124.63	mEq/L	. - .

Reference Range

AGE IN YEARS

Males - 25-301 Females--25-267

Clinical significance :

Sodium (Na+) is the primary extracellular cation. Na+ is responsible for almost one-half the osmolality of the plasma and, therefore, plays a central role in maintaining the normal distribution of water and the osmotic pressure in the extracellular fluid compartment. The amount of Na+ in the body is a reflection of the balance between Na+ intake and output. The normal daily diet contains 8 to 15 grams of sodium chloride (NaCl), which is nearly completely absorbed from the gastrointestinal tract. The body requires only 1 to 2 mmol/day, and the excess is excreted by the kidneys, which are the ultimate regulators of the amount of Na+ (and thus water) in the body. Na+ is freely filtered by the glomeruli. Approximately 70% to 80% of the filtered Na+ is actively reabsorbed in the proximal tubules with chloride and water passively following in an iso-osmotic and electrically neutral manner. Another 20% to 25% is reabsorbed in the loop of Henle along with chloride and more water. In the distal tubules, interaction of the adrenocortical hormone aldosterone with the coupled sodium-potassium and sodium-.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MRS. RASHIDA BANO ANSARI**
Ref By : **DR. VIKRAM B. JAIN.**
Reg No : **HL5901823158 / RAJ41/RAY07**
Barcode : **HEL096707**
INV : SPUTUM For Koh

Age : 67 Year | **Sex :** Female
Registered on : 22-08-2026 09:36 PM
Received on : 23-08-2026 03:55 AM
Reported on : 23-08-2026 11:09 PM

SAMPLE COLLECTED AT :
SUN LABORAOTRY

SAMPLE : SPUTUM

SPUTUM FOR KOH

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SPUTUM FOR KOH	NEGATIVE	.	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Age : 67 Year | **Sex :** Female
Registered on : 22-08-2026 09:36 PM
Received on : 23-08-2026 03:55 AM
Reported on : 23-08-2026 11:09 PM

SAMPLE COLLECTED AT :
SUN LABORAOTRY

SAMPLE : SPUTUM

SPUTUM FOR KOH

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SPUTUM FOR KOH	NEGATIVE	.	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MR.NILESH JADHAV**	**Age :** 39 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** DR DIAGNOSIS PATHOLOGY LAB
Ref By	: **DR. SELF**	**Registered on :** 25-08-2026 11:21 AM	
Reg No	: **HL1701825629 / HYMH/NALSPR/H7643**	**Received on** : 28-08-2026 10:40 AM	
Barcode	: **HEN111204**	**Reported on** : 31-08-2026 07:34 PM	**SAMPLE :** Stone
INV	: Stone Analysis Kidney		

STONE ANALYSIS KIDNEY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Physical Examination			
Colour	Brownish		
Appearance	stone with rough surface.		
Weight	0.5gms Grams -		
Biochemical Examination			
Calcium	Present (25%) % -		
Oxalates	Present (20%) % -		
Phosphates	Present (20%) % -		
Magnesium	Present 30 % -		
Uric Acid	Present (05%)-		

~~End of report~~

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REPORT

PT Name : **SANGEETA KOILKAR**
Ref By : **DR. SELF**
Reg No : **HL3801829851 / MH031**
Barcode : **HEN199498**
INV : Synovial Fluid For Crystals

Age : 48 Year | **Sex :** Female
Registered on : 29-08-2026 09:38 AM
Received on : 29-08-2026 04:30 PM
Reported on : 31-08-2026 11:03 PM

SAMPLE COLLECTED AT :
SHRIKANT MURCHAVDE

SAMPLE : Body Fluid

SYNOVIAL FLUID FOR CRYSTALS

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Synovial Fluid for Crystals Method: .	NEGATIVE	.	. - .

~~End of report~~

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REPORT

PT Name : **MINAKSHI MAHAJAN**
Ref By : **DR. NAMCO HOSPITAL**
Reg No : **HL7901828417 / MH394**
Barcode : **8451784102**

INV : Thyroglobulin

Age : 40 Year | **Sex :** Female
Registered on : 27-08-2026 03:10 PM
Received on : 28-08-2026 01:16 AM
Reported on : 28-08-2026 07:24 AM

SAMPLE COLLECTED AT :
NISHKARSH CLINICAL LABORATORY , MANMAD, NASHIK, DR BHARAT JAGTAP. MOBAIL NUMBER-- 9860338188

SAMPLE : Serum

THYROGLOBULIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
THYROGLOBULIN	1.26	IU/mL	0.83 - 68.0

Method: Immunoenzymatic Assay

Note

1. This test is not recommended for the diagnosis of Thyroid cancer
2. Presence of Thyroglobulin autoantibodies can interfere in the assay leading to an under estimation of Thyroglobulin levels. Therefore the Thyroglobulin antibody status should be determined when measuring Tg
3. Minimum 6 weeks should elapse post thyroidectomy or thyroid gland ablation prior to testing
4. Thyroglobulin levels may remain elevated for several months following successful cancer therapy. In these cases post treatment baseline values followed by serial determinations are recommended for monitoring

Clinical Use:

* Primarily used as a tumor marker in patients with a diagnosis of Differentiated Thyroid carcinoma (DTC). Tg levels are elevated in both Thyroid, Papillary & Follicular carcinoma
* Serial measurements of Tg are most useful in detecting recurrence of DTC following surgical resection or radioactive Iodine ablation
* Aids in the management of infants with Congenital Hypothyroidism
* Differential diagnosis of Hyperthyroidism. Tg is elevated in all patients with Hyperthyroidism except in cases of Thyrotoxicosis factitia

Increased Levels:

* Thyroid Papillary & Follicular carcinoma
* Non neoplastic thyroid conditions like Thyroid adenoma, Subacute thyroiditis, Hashimoto's thyroiditis & Graves' disease * Regions of Endemic goitre
* Neonates
* Third trimester of pregnancy

~~End of report~~

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MR. CHAMPALAL KAYASTH**	**Age :** 75 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** BIOPATH CLINICAL LABORATORY
Ref By	: **DR. BIOPATH CLINICAL LABORATORY**	**Registered on :** 28-08-2026 04:17 PM	
Reg No	: **HL9001829468 / MH004/MS102/MS208**	**Received on** : 29-08-2026 01:50 AM	
Barcode	: **HEB052048**	**Reported on** : 29-08-2026 12:26 PM	
INV	: Transferrin		**SAMPLE :** Serum

TRANSFERRIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TRANSFERRIN, SERUM	**10.2**	.	13 - 45

Note
Pregnancy and use of oral contraceptive cause increase in transferrin levels

Comments
Transferrin is a transport protein which transfers ferric iron from iron stores to bone marrow. In response to short term iron deficiency, transferrin levels rise markedly to twice the normal levels or higher. An elevated level on electrophoresis can have the appearance of a paraprotein in cases of severe iron deficiency. Administration of iron to deficient patients increases the saturation followed by return of transferrin to normal level. Chronic saturation of transferrin occurs in Idiopathic hemachromatosis and Transfusional hemosiderosis. In severe cases of Protein losing nephropathy, transferrin is lost from the circulation in the urine carrying iron with it, thus leading to Hypochromic anemia. Simultaneous measurement of transferrin with ferritin helps to differentiate anemia due to iron deficiency and chronic inflammation.

~~End of report~~

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REPORT

PT Name : **MRS.JAIBUNNISA ANSARI**
Ref By : **DR. SELF**
Reg No : **HL6901832485 / MH249**
Barcode : **HEN158600**
INV : TSH Ultrasensitive

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 05:48 PM
Reported on : 31-08-2026 06:56 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

TSH ULTRASENSITIVE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TSH (Ultrasensitive) Method: ECLIA	1.53	uIU/ml	. - . Hyperthyroid < 0.270 Euthyroid 0.270 - 4.200 Hypothyroid > 4.200

Interpretation :

Thyroid Stimulating Hormone is a glycoprotein with two – covalently bound subunits.The alpha subunit is similar to those of follicle stimulating hormone (FSH), human chorionic gonadotrophin (hCG) and Luteinizing Hormone (LH).The beta submit of TSH is unique, which results in the specific biochemical and immunological properties of this hormone.TSH is synthesized and secreted by the anterior pituitary in response to a negative feedback mechanism involving concentrations of FT3 (Free T3) and FT4 (Free T4).Additionally, the hypothalamic tripeptide, thyrotropoin – releasing hormone (TSH), directly stimulates TSH production.TSH interacts with specific cell receptors on the thyroid cell surface and exerts two main actions.The first action is to stimulate cell reproduction and hypertrophy.Secondly,TSH stimulates the thyroid gland to synthesize and secrete T3 and T4 .In primary
hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism TSH levels are low.
Below mentioned are the guidelines for pregnent woman related reference ranges of TSH recomemded by Americal Thyroid Association:

Pregnancy	uIU/mL
First Trimester	0.600 - 3.400
Second Trimester	0.370 - 3.600
Third Trimester	0.380 - 4.040

~~End of report~~

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